



# Fiona Stanley Fremantle Hospitals Group

## Clinical risk: assessment and management

Reference #: FSFH-HW-POL-0017

### Scope

| Site                                                            | Service/Department/Unit | Disciplines                              |
|-----------------------------------------------------------------|-------------------------|------------------------------------------|
| Fiona Stanley Hospital<br>Fremantle Hospital<br>Cockburn Health | Mental Health Service   | Medical, Nursing and Allied Health staff |

### 1. Introduction

Even with the best clinical risk assessment practices and care it is not possible to prevent or eliminate all risk. Assessing risk is an essential first step to establishing and agreeing on the appropriate interventions required to minimise the likelihood of harm to the consumer or others. Clinical risk assessment and management will focus on the consumer's individual history and current circumstances and on risk mitigation. This policy and procedure outlines the mental health services standards for clinical risk assessment and management.

### 2. Terminology

|                        |                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Clinical risk</b>   | In mental health is defined as the likelihood of an event occurring, which may have harmful outcomes for the consumer or others.                                                                                                                                                                                                                                                                                                          |
| <b>Risk assessment</b> | The gathering and analysis of clinical information related to risks to the consumer and/or others. It includes background (static) risk factors interacting with acute (dynamic) risk factors and the consumers current diagnosis and Mental State Examination to provide a risk assessment framework for clinical judgment and decision making. An accurate assessment of risk cannot be established until safety planning is completed. |
| <b>Safety Planning</b> | A structured plan developed collaboratively between health professionals, the consumer, their family /significant other and other culturally appropriate health support workers. It describes strategies to identify, manage and reduce risk and enhance safety, including actions to be taken, when and by whom.                                                                                                                         |

|                                                                |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                            |
|----------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Risk formulation</b>                                        | <p>The analysis and summary of risk information which includes longitudinal, unaddressed and cross-sectional risk gathered from comprehensive mental health and risk assessment into a formulation which assesses the patients available internal and external resources and foreseeable changes that may lead to an increase in risk.</p> <p>The formulation may include risk benefit analysis, consultation with colleagues and communication with support persons.</p>                                  |
| <b>Longitudinal risk</b>                                       | Compared to the patient' timeline of risk. Consider where the consumers risks is related to their baseline, their worst point and best point.                                                                                                                                                                                                                                                                                                                                                              |
| <b>Unaddressed risk</b>                                        | Risk assessments must occur in parallel with safely planning. Identified risks require safety planning interventions and risks that cannot be mitigated after safety planning are referred to as unaddressed risks. When risks are unable to be adequately addressed it may lead to the identification of clear and imminent risk.                                                                                                                                                                         |
| <b>Cross sectional risk</b>                                    | Compared to other patient populations. Consider if the consumers risk is lower or higher than an inpatient, Hospital in the Home patient, community patient or under primary care.                                                                                                                                                                                                                                                                                                                         |
| <b>Mental State Examination (MSE)</b>                          | A structured psychiatric assessment divided into categories of appearance, behaviour, motor activity, speech, mood, affect, thought process, thought content, perceptual disturbances, cognition, insight, and judgment. This information is used as part of the diagnosis and monitoring of mental illness and risk.                                                                                                                                                                                      |
| <b>Structured clinical judgement</b>                           | <p>Structured clinical judgment in relation to assessing and managing risk involves the clinician making a judgement about risk based on an individual consumers assessment including the consumer's own view of their experience and circumstances, collaboration with family/significant others together with clinical experience and knowledge of the consumer and their history.</p> <p>Structured clinical judgement is made in collaboration and consultation with the wider mental health team.</p> |
| <b>Psychiatric Services Online Information System (PSOLIS)</b> | Statewide mental health clinical information system used by medical, nursing, allied health, management and administration staff within public Mental Health Services. It provides clinicians with the ability to share, analyse and retrieve consumer information.                                                                                                                                                                                                                                        |
| <b>Care Coordinator</b>                                        | Mental health clinician nominated the role of managing a consumer's care. They monitor and coordinate consumers treatment plans, provide psychoeducation to the consumer and family/significant others, connect them with health care                                                                                                                                                                                                                                                                      |

providers, and evaluate their progress in consultation with the Multidisciplinary Team (MDT).

### 3. Policy

- Any clinical professional involved in consumer care must complete a risk assessment when clinically indicated.
- Active and empathic listening can promote rapport and therapeutic alliance and enhance clinical risk assessment.
- Documentation of clinical risk assessments must include an action plan to mitigate risks.
- Information must be sought from family/significant others, support networks and previous care providers, with the consumer's permission where possible.
- The practice of risk assessment and management will be person-centred, recovery oriented, developmentally appropriate, culturally safe, trauma informed and completed in collaboration with MDT oversight.
- If domestic abuse is identified, the safety and wellbeing of any children involved must be considered a priority.
- Consumers must be provided access to clinical staff of the gender of their choice if requested and wherever possible.
- Risk assessment and management will acknowledge the balance of risks with consumer choice and dignity. Where this is not possible the reasons must be documented in the medical record.
- Clinicians will aim to apply risk management plans and actions in a manner that is least restrictive for the consumer while balancing safety for consumers, staff and the community.
- An assessment with consumers who identify as Aboriginal or Torres Islander must be undertaken in a culturally safe manner and in accordance with the Mental Health Act (MHA) 2014. Aboriginal or Torres Islander consumers will be referred to Aboriginal Liaison Services

### 4. Procedure

#### 4.1. Risk factors

- Factors that may increase a consumer's risk include, but are not limited to:
  - Vulnerable and marginalised groups
  - Aged under 18 or over 65 years of age, especially those with parents or guardians with limited ability for safeguarding.
  - Cognitive impairment and or psychiatric illness
  - Personality structure

- Disinhibition
- Trauma history
- Family and domestic violence
- Spiritual and cultural diversity
- Substance/alcohol use
- Absence of support network

## 4.2. Risk types

Risks that must be assessed and managed include, but are not limited to:

- Risk to self:
  - self-harm and suicide,
  - self-neglect
  - absconding and wandering
  - quality of life, social and financial status, homelessness
- Physical health:
  - alcohol and substance use and withdrawal.
  - neglect of acute and chronic medical conditions
  - side-effects of medication e.g. metabolic syndrome
  - nicotine withdrawal
  - falls risk.
- Risk to others:
  - harassment, stalking, predatory intent
  - violence and aggression, sexual assault or abuse
  - property damage
  - public nuisance
  - reckless behaviour
- Risks from others:
  - physical, sexual, emotional abuse and or coercive control
  - social abuse or neglect
- Environmental/functional risk:
  - Not able to be managed safely in the community.
  - In the inpatient setting consider the ward environment

## 4.3. Risk Assessment

- A review of the consumers risks will be undertaken at each clinical contact and documented in the medical record with an MSE.
- Significant risks must be documented in PSOLIS alerts and will be incorporated into the Treatment Support and Discharge Plan (TSDP).

- A formal clinical risk assessment will be documented on a Risk Assessment and Management Plan (RAMP) in webPSOLIS and uploaded to the consumer's medical record:
  - at first contact with the Mental Health Service if clinically indicated otherwise at first formal assessment
  - on admission to and discharge from an inpatient mental health unit
  - at the time of MH assessment in the Emergency Department (if not already completed).
  - on discharge from the service or transfer of care to another service
- A RAMP or a structured entry in the medical record titled risk assessment with management strategies will be documented for the following events:
  - Increase in clinical acuity related to:
    - consumers clinical presentation
    - identified risk factors.
    - known times of high risk in relation to the individual consumer
    - the consumer's social situation
  - When a consumer is transferred between wards
  - Change in status under the MHA 2014
  - Prior to and return from inpatient leave irrespective of MHA 2014 status
  - Following incidents related to behaviour or mental state.
  - When concerns are reported by staff or family/significant others
  - Clinical review meeting/intake meeting/ward round
- Assessments, wherever practicable and appropriate to do so, will be undertaken in collaboration with:
  - Aboriginal Mental Health Liaison staff and/or significant members of the consumers community including Elders and Traditional Healers.
  - For patients who have communication barriers interpreter services and alternative modes of communication are to be considered

#### 4.4. Risk management

- Clinicians will consider the consumer's history, the identified risk factors and use structured clinical judgement to analyse the identified risks and document a risk formulation which may include longitudinal risks, unaddressed risks and cross-sectional risks.
- Prioritise the actions required to manage risks according to:
  - if risk not reduced or mitigated, significant potential for an adverse outcome for the consumer/other.
  - if risk not reduced or mitigated, unlikely for there to be an adverse outcome for the consumer/other.
- Management of clinical risks includes:

- identifying actions likely to reduce, or eliminate fully, identified risks in a clinically appropriate manner and not cause further harm.
  - accepting those risks determined by the MDT as appropriate to accept according to the patient's current clinical condition.
- Formulating a risk management plan and individualized safety plan incorporating identified options/actions and details of accountability to ensure agreed steps are undertaken.
- The safety plan must systematically address identified risks and appropriate escalation must occur if residual unaddressed risks remain after safety planning.
- Collaboratively share and plan care with the consumer and, where appropriate, family/significant other and wider team of key stakeholders which may include NDIS, Corrective Services, AOD services, Department of Communities and non-government organisations.
- Incorporate appropriate elements from the risk assessment and management plan into the consumers TSDP in webPSOLIS and ensure a copy is made available in the medical record and provided to the consumer and their family/significant other.
- Monitor the consumer to ensure the risk management plan is effective and collaborative reviews occur with the patient and relevant key stakeholders to ensure the plan is appropriate and meaningful.
- Reflect changes in the consumer's risk profile and interventions required chronologically in the TSDP.
- Communicate identified clinical risks and the management plan at each clinical handover and clinical review with the treating team.
- Escalate risks if unable to be mitigated or reduced, these risks must be brought to the immediate attention of the treating team for Consultant Psychiatrist review.

## 5. Compliance Monitoring

The Service 5 Safety Quality and Risk Committee conducts annual audits on Sexual safety in the inpatient units, access to means of self-harm, absent without leave and aggressive incidents.

The Service Director will monitor workplace safety and health incidents related to behaviour incidents.

## 6. Related Policy Documents

Department of Health:

- [Safety Planning for Mental Health Consumers Policy MP 0181/245](#)

- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and best practice for the clinical care of people who may be suicidal](#)
- [Principles and best practice for the clinical care of people who may be at risk of exhibiting violent or aggressive behaviour](#)
- [Chiefs Psychiatrists Sexual Safety Guidelines](#)
- [State-wide Standardised Clinical Documentation for Mental Health Services](#)

SMHS:

- [SMHS Care Coordination in Mental Health](#)
- [SMHS Staying Safe Pathway](#)

FSFHG:

- Clinical Documentation (FSFH-HW-POL-0039)
- Clinical Handover (FSFH-HW-POL-0035)
- Clinical Care of People who may be Suicidal (FSFH-HW-POL-0037)
- Clinical Handover (FSFH-HW-POL-0035)
- Family and Domestic Violence (FSH-HW-PRO-0007)
- Sexual safety in Mental Health (FSFH-MENH-POL-0012)

## 7. Related documents

[The Western Australia Mental Health Act 2014](#)

## 8. Related Standards

NSQHS Standards

- Partnering with Consumers
- Comprehensive care
- Communicating for safety
- Recognising and responding to acute deterioration

Chief Psychiatrists Standards for Clinical Care:

- Consumer and carer involvement in individual care
- Risk assessment and management
- Care planning

## 9. Authorisation

| EXECUTIVE SPONSOR: Service Director, Service 5 |             |                           |                                    |                                              |              |
|------------------------------------------------|-------------|---------------------------|------------------------------------|----------------------------------------------|--------------|
| Version                                        | Date Issued | Compiled/Revised By       | Committee/Consumer Group Consulted | Endorsed By                                  | Revision due |
| 1                                              | 02/14       | Clinical Co Director      | FH MHS CGC                         | PLCC                                         | 02/2017      |
| 2                                              | 02/16       | CN                        | FH MHS CGC                         | PLCC                                         | 03/2020      |
| 3                                              | 07/20       | CNS, Clinical Improvement | Service 5 SQR Committee            | FSFHG Policy Committee                       | 12/2021      |
| 4                                              | 03/22       | CNS, Clinical Improvement | NMPGC                              | Service 5, Safety Quality and Risk Committee | 03/2024      |
| 5                                              | 07/24       | CNS, Clinical Improvement | NMPGC                              | Service 5, Safety Quality and Risk Committee | 08/2027      |